Royal Devon University Healthcare NHS Foundation Trust

Resuscitation Policy	
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KEY POINTS OF THIS POLICY:

Healthcare organisations have an obligation to provide a high-quality resuscitation service and ensure that staff are trained and updated with appropriate frequency to a level of proficiency appropriate to each individual's expected role (Resuscitation Council UK).

This policy provides staff with guidance on the responsibilities of the organisation, resuscitation service and staff in the recognition, escalation and management of deteriorating patients and in the management of a cardiopulmonary arrest. This guidance is as recommended by the Resuscitation Council (UK). This policy applies to all staff within the organisation in the acute and community setting.

The Key points within this policy are regarding

- Recognition and escalation of deteriorating patients in the acute and community setting using Early Warning Scores.

- What to do in the event of a cardiopulmonary arrest.

  All patients, visitors and staff within the Trust will receive resuscitation in the event of a cardiopulmonary arrest unless they have been designated 'not for an attempt at cardiopulmonary resuscitation' by their medical team (Treatment Escalation Plan [TEP] in place) or have an appropriate advanced directive.

  The treatment carried out during resuscitation is that recommended by the Resuscitation Council (UK) and the European Resuscitation Council, whose guidelines are followed for adult, paediatric and neonatal resuscitation.

  As a minimum, patient-facing staff will be able to recognise cardiac arrest and summon help. They will instigate Basic Life Support as per current Resuscitation Council (UK) guidance and if appropriate deliver a shock using an Automated External Defibrillator (AED).

- Education and training. This policy applies to all staff working within the Trust. They are responsible for ensuring they attend the required level of training and required updates identified for their staff group, in conjunction with their line manager.

- Resuscitation equipment. There should be standardisation of equipment across sites and departments to facilitate use in medical emergencies. Resuscitation trolleys and grab bags will be standardised, accepting that some areas will require adjustments. Adjustments will only be made by the Resuscitation Service. Some areas may have equipment over and above standard equipment. In general, equipment which is used only by specialist staff should be brought with those experts when requested.

1 INTRODUCTION

1.1 Resuscitation is the process of providing medical intervention to rapidly deteriorating and/or critically unwell patients. This includes the identification and assessment of patients of all ages who are deteriorating due to illness or injury, as well as those who have suffered a respiratory or cardiorespiratory arrest. Resuscitation may include administration of medications, surgical or other invasive procedures, mechanical organ support such as ventilation, and other interventions. Cardiopulmonary resuscitation, and Advanced Life Support, refer to specific treatments for patients in cardiac arrest.

1.2 The process of resuscitation is therefore not solely applicable to patients in cardiac arrest, and is carried out across the range of healthcare settings in response to sudden deterioration. This includes emergency care, critical (intensive) care, and inpatient settings, but also outpatients, community sites, and outside of the healthcare environment.

1.3 All patient-facing staff, throughout the Royal Devon University Healthcare NHS Foundation Trust, in acute and community settings, should be trained to identify a patient who is collapsed or deteriorating, and to respond appropriately to that deterioration. The level of training and expected response will depend on the individual and the role they are employed to do.

1.4 Cardiopulmonary resuscitation should be performed on any individual in the event of cardiopulmonary arrest unless there is a TEP or an Advanced Decision to Refuse Treatment stating that the individual is not for resuscitation, unless a senior clinician identifies that this is medically futile. All clinical staff and some non-clinical staff with patient contact will be trained to a minimum of Basic Life Support as recommended by current Resuscitation Council (UK) Guidance.

2 PURPOSE

2.1 The purpose of this policy is to support the delivery of a high-quality resuscitation service to the Trust (acute and community), including recognition of and response to deteriorating patients, and training requirements for staff.

2.2 The policy incorporates the current published guidelines for resuscitation by the Resuscitation Council UK.

3 DEFINITIONS

3.1 **ADRT** – Advanced Decision to Refuse Treatment. Formerly known as Advance Directives or Living Will

3.2 **AED** – Automated External Defibrillator is a portable device that analyses the heart rhythm and can deliver an electric shock to the heart if necessary

3.3 **ALS** – Advanced Life Support refers to emergency medical procedures in which basic life support efforts of cardiopulmonary resuscitation are augmented by more advanced medical interventions. This may include establishment of an intravenous (IV) fluid line, defibrillation, drug administration, control of cardiac arrhythmias, airway management skills, and use of ventilation equipment. This term also refers to a Resuscitation Council accredited course on which these skills are taught

3.4 **CPR** – Cardiopulmonary Resuscitation is an emergency procedure in which the heart and lungs are made to work by manually compressing the chest overlying the heart and forcing air into the lungs. CPR is used to maintain circulation when the heart stops pumping

3.5 **BLS** – Basic Life Support refers to maintaining airway patency and supporting breathing and the circulation without the use of equipment other than protective devices

3.6 **BPA** – Best Practice Advisory, a pop-up alert on Epic

3.7 **DART** – Deterioration and Resuscitation Team (Northern)

3.8 **DNACPR** – Do Not Attempt Cardiopulmonary Resuscitation in the event of the person suffering a cardiac arrest

3.9 **Epic** – Electronic patient record

3.10 **ILS** – Immediate Life Support is a Resuscitation Council UK accredited course

3.11 **IMCA** - Independent Mental Capacity Advocate is a role created by the Mental Capacity Act 2005. A local council or NHS body has a duty to involve an IMCA when a person aged ≥16 who lacks mental capacity to understand a decision which needs to be made about serious medical treatment, and has no suitable person to represent them. The IMCA will represent the patient and act in the person's best interests

3.12 **LPA** – Lasting Power of Attorney. A person nominated to make decisions on behalf of a person if they are unable to make decisions due to lack of capacity

3.13 **MET** – Medical Emergency Team (Eastern)

3.14 **NEWS2** – National Early Warning Score version 2 is a score calculated every time a patient's physiological observations are undertaken to help identify early signs of deterioration

3.15 **RO** – Resuscitation Officer

3.16 **ROSC** - Return of Spontaneous Circulation following a cardiac arrest

3.17 **TEP** – Treatment Escalation Plan details the resuscitation status and the scope of appropriate care for patients

4 DUTIES AND RESPONSIBILITIES OF STAFF

4.1 The **Chief Executive** has overall responsibility for the effective implementation of this policy throughout the Trust.

4.2 The **Patient Safety Committee** are responsible for receiving assurance from the Patient Safety Operational Group on all resuscitation matters. The committee will ratify the policy and ensure policy distribution, implementation and compliance throughout the organisation.

4.3 The **Patient Safety Operational Group** reports to the Patient Safety Committee. The PSOG will approve an annual Patient Safety Programme, and seek assurance and advice from the Resuscitation Group about all resuscitation matters

4.4 The **Resuscitation Group** has overall responsibility for the implementation, evaluation and monitoring of the resuscitation policy throughout the Trust and will report to PSOG

The Resuscitation Group must include representatives from stakeholder groups, as recommended in the quality standards for cardiopulmonary resuscitation practice and training in the acute care setting, (Resuscitation Council, 2020)

The Chair of the Resuscitation Group must be a senior clinician with an active and credible involvement in resuscitation. This individual would be expected to have the authority to drive and implement change.

The role of the resuscitation group is to:

- Oversee policy and guideline development
- Advise the Trust on resuscitation policy, process and equipment in line with the Resuscitation Council UK guidelines
- Review equipment and procedures for resuscitation, and ratify change
- Receive reports on training compliance
- Review audits on deterioration, resuscitation and equipment
- Identify and escalate specific risks
- Review trends and work with the governance and safety teams to improve care

The group will meet quarterly to achieve the aforementioned duties, with the appropriate administrative support required.

4.5 **Resuscitation Officers** (ROs) are responsible for the resuscitation service and will ensure effective and efficient delivery of resuscitation training throughout the Trust

4.5.1 The ROs are responsible for the management of the public access defibrillator cabinets (Eastern Services)

4.5.2 They will audit and maintain a database of all cardiac arrest calls and submit data to the NCAA. They will endeavour to collect data on 2222 calls but serious incidents should be reported via the Datix system as per the Trust policy.

4.5.3 The ROs will ensure standardisation of resuscitation equipment across the Trust, recognising that individual areas may require specific adjustments to meet their needs.

4.5.4 The Resuscitation Officers will provide first contact for those affected by cardiac arrest (including patients, co-survivors, relatives, staff, and others), signposting them to other services as necessary

4.6 The **Cardiac Arrest Team Leader** will coordinate and run the resuscitation attempts using the Resuscitation Council guidelines (https://www.resus.org.uk/)

4.7 **Ward and Departmental Managers** have overall responsibility for implementing this policy within their area(s) of control.

4.8 The **Medical Equipment Management Manager (MEM)** is responsible for the maintenance and checking of equipment and should be consulted prior to new equipment being purchased for the Trust.

4.9 **All patient-facing staff** have a responsibility to update their Basic Life Support skills annually and attend intermediate or advanced courses if identified as required for their role. All clinical staff also have a responsibility for ensuring that any resuscitation equipment is ready for use at all times and highlighting any issues they may identify.

5 EDUCATION AND TRAINING

5.1 All training will be delivered following current Resuscitation Council (UK) standards and guidelines.

5.2 The Trust has formulated a matrix for guidance on required resuscitation training (including update timeframes) for all Trust staff. This is based on the Core Skills Training Framework and will be reviewed by the Resuscitation Group and updated by the Senior Resuscitation Officers.

5.3 Formal resuscitation training events will be recorded, with all documented attendees being submitted for inclusion on the Learn+ database – staff must ensure they remain compliant with training via Learn +

5.4 Patient-facing staff who are new to the Trust must receive face-to-face resuscitation training during their Trust induction. All visiting patient-facing staff (including students, agency etc.) should be trained as for substantive staff in the same role, by their responsible organisation (employer, university, etc.), prior to commencing activities with patients.

5.5 Staff should NOT withhold administering CPR if their current life support certificate is ‘out of date’. Staff with prior instruction in resuscitation should administer CPR to their current scope of knowledge.

6 RECOGNITION AND ESCALATION OF THE DETERIORATING PATIENT

6.1 Early warning scores

6.1.1 The Trust utilises the National Early Warning Score (NEWS2) system of monitoring of adult patients’ vital signs to identify patients at risk of deterioration (Appendix D).

6.1.2 A NEWS2 Score should be calculated within Epic every time an adult patient's physiological observations are undertaken; this includes acute and community sites, those attending the Emergency Department, and any procedure where clinical observations are taken beforehand. Patients being treated in their own homes for acute illness, or where there is clinical concern, should have a NEWS2 score calculated.

6.1.3 Paediatric patients should be scored similarly but using the specific Paediatric Early Warning Score (PEWS) system based on their age.

6.1.4 Pregnant, or recently pregnant, patients should have their score calculated using the Maternal Early Warning Score (MEWS). Pregnant patients being cared for outside maternity environments will have both NEWS2 and MEWS scores calculated in Epic.

6.1.5 Caution should be exercised using the NEWS2 score for patients with a spinal cord injury.

6.1.6 All clinical staff involved in recording and interpreting patient observations should be appropriately trained in both the technical skills to perform them, and the interpretation within the Early Warning System. The Trust is responsible for ensuring that all clinical staff undertaking observations are appropriately trained to do so.

6.1.7 Deviations from normal physiological parameters generate a score, which will trigger intervention from medical and nursing staff as detailed on the BPA within Epic.

6.1.8 All clinical staff must be aware of their role and responsibility in the effective recognition and escalation of a patient who may be deteriorating.

6.1.9 The patient's EWS should be included as part of the formal handover of patients. Patients being transferred within the Trust should have their EWS calculated prior to moving and again on arrival in the new area.

6.1.10 The frequency of when observations are carried out and recorded should follow the relevant EWS recommendation in accordance to the patient's condition. There may be circumstances when clinical judgement overrides this recommendation, and in this situation, it may be appropriate to increase frequency of observations. The minimum frequency of observations in hospital is at least 12-hourly unless the patient is receiving end of life care where it should be documented in the patient's plan 'not for observations' – this can be performed in the End of Life navigator in Epic. If the patient is at ceiling of care on the wards, then observations need not be performed so frequently. This will not prevent the nurses performing observations if they are concerned. Doctors should write a clear management plan that stipulates the frequency of observations and when they feel further escalation should occur.

6.1.11 Some patients require alternative targets for oxygen saturations due to the risk of hypercapnia – oxygen 'scale 2'. The decision to place patients on scale 2 should only be made by a senior clinician with documented evidence of hypercapnic respiratory failure (based on a blood gas from previous or same hospital admission).

6.1.12 Adapted parameters can be documented by a senior doctor. For example, if the patient has a condition which deviates from the considered normal range such as oliguric renal failure, or chronic respiratory disease, it may be appropriate to document an acceptable deviation. If the EWS parameters are altered, it is important that they are reviewed frequently, and a plan made to re-adjust to the normal parameters when/if this is clinically appropriate.

6.1.13 Where a patient is recognised as being in the last few days or hours of life a personalised plan of care should be made and agreed by the patient, family and multi-disciplinary team using the dedicated documentation.

6.1.14 Regardless of EWS clinical staff should continue to use their clinical judgement when making decisions around the monitoring, escalation and treatment of a deteriorating patient. All staff should feel empowered to summon emergency help if they are concerned about a patient.

6.2 Acute Eastern Services

6.2.1 The Eastern site uses a Medical Emergency Team (MET) to respond to acute clinical emergencies in adult patients identified by clinical triggers (NEWS2 score) or other indicators. (Contacted via 2222 asking for MET attendance).

6.2.2 The MET is made up of the following personnel:

- Medical registrar (ST3 / IMT3 or above, or equivalent)
- On call ICU – variable profession and seniority, but always with access to a senior ICU opinion either via Consultant (during the day) or by 4th on call anaesthetist at night.
- Medical F2 on call
- Senior Matron on call during working hours
- Site Practitioner (out of normal working hours, for any incident outside the main building, and to any in-hours on-site incident if available)
- Resuscitation Officer (during normal working hours, if available)

6.2.3 The MET will assist the patient's own team to plan further care for the patient, and will provide immediate supportive care as appropriate. If a member of the patients own team does not attend the MET call, the staff on the ward must ensure that they are contacted (or the responsible on-call team out of hours).

6.2.4 The Medical Emergency Team (MET) is based only in the Wonford Main Building, for all other sites, if appropriate, call for an emergency ambulance, ([9]999).

6.2.5 If a patient deteriorates rapidly or collapses staff should summon immediate help by dialling 2222 and asking for either the cardiac arrest team or MET as appropriate.

6.3 Acute Northern Services

6.3.1 The site has a Deterioration and Resuscitation Team (DART) which are available every day 08:00 – 18:30, to assist in the management of acutely deteriorating patients. They can be contacted on bleep 007.

6.3.2 The DART will assist the patient's own team to plan further care for the patient, and will provide immediate support as appropriate.

6.3.3 The team is based only in the NDDH Main Building; for all other sites, if appropriate, call for an emergency ambulance ([9]999).

6.3.4 If a patient deteriorates rapidly or collapses, Staff should summon immediate help by dialling 2222 and asking for the cardiac arrest team.

6.3.5 Outside of DART operational hours, summon help from the patient's medical team. In an emergency dial 2222 and ask for the cardiac arrest team.

6.4 Community sites

6.4.1 Adult patients in community hospitals should have their observations recorded and scored using the NEWS2 system, as outlined in section 6.1 above.

6.4.2 Where clinical observations are taken in other community settings these should also be calculated into a NEWS2 score.

6.4.3 If a patient is deteriorating or has a high NEWS2 score escalation should be in line with the community deterioration guidance (Appendices E-G) and an emergency ambulance called using (9)999 if appropriate. In a patient's home, a high NEWS2 score should be immediately escalated to the responsible clinician, with an emergency ambulance called if necessary. Domiciliary teams should follow the community guidelines for managing deteriorating patients.

6.4.4 Initiation of the appropriate emergency protocols / interventions must occur while awaiting ambulance arrival.

6.4.5 Any interventions and relevant history must then be handed over to ambulance personnel.

6.4.6 Where resources allow and clinical need indicates, it may be necessary for a member of the team to accompany the patient to the acute hospital. Where there is a need for a level of care for transfer which is not immediately available (for example, ongoing general anaesthesia), clinicians should contact RETRIEVE for urgent advice and support (0300 030 2222).

6.4.7 Staff at The Nightingale Hospital Exeter should follow the Nightingale Hospital Exeter escalation policy: Escalation Policy and Off-Pathway Transfers SOP.

7 RESUSCITATION EQUIPMENT

7.1 The Resuscitation Group will set the parameters for equipment in each of these areas guided by the minimum equipment and drug lists for cardiopulmonary resuscitation (Resuscitation Council, 2020). Appropriate adjustments to equipment lists may be made if required for specific clinical areas.

7.2 The Resuscitation Officers will ensure that resuscitation equipment is up to date and fit for purpose by carrying out audits of equipment and local checks.

7.3 All resuscitation trolleys and grab bags should be stocked in accordance with the latest approved checklist. The resuscitation team will provide approved checklists for all areas throughout the organisation using the current checking system.

7.4 All resuscitation trolleys and grab bags must be maintained in a state of readiness at all times.

7.5 It is the responsibility of all staff working in an area to familiarise themselves with the location of all resuscitation equipment.

7.6 Monthly checks of the contents of the resuscitation trolley/grab bag should be carried out by two members of staff one of whom must be a registered nurse/practitioner and a unique numbered seal applied.

7.7 Daily checks are required for defibrillators, oxygen cylinders and suction and that the trolley seal has the correct number on it when a room or bed space is being used.

7.8 Following the conclusion of a resuscitation attempt or if equipment has otherwise been used from the resuscitation trolley/grab bag, the trolley/grab bag should be checked, restocked and a unique numbered sealed tag applied.

7.9 Ward/department managers are responsible for ensuring that the trolley/grab bag checks are performed and for taking appropriate action if this has not been done.

7.10 Equipment Replacement

7.10.1 **Eastern Acute** – Replacement items should be obtained via the area ‘top-up’ store or for specialised equipment from the emergency equipment cupboard located outside General Theatres. Site practitioners can be contacted out of hours.

7.10.2 The General Theatre receptionist holds the key to the cupboard in normal working hours; for out of hours contact, the theatre duty senior nurse (Bleep 801) or site practitioner.

7.10.3 **Eastern Community** – Replacement items can be obtained by emailing the Resuscitation Officers generic email (rduh.resusofficers@nhs.net) who will then post the items requested.

7.10.4 **Northern Acute** – Replacement items should be obtained either from ward stock, from the top-up cupboard opposite Switchboard or, for specialist items, from theatre stores.

7.10.5 **Northern Community** – Replacement items should be obtained by scanning the QR code on the Grab Bag to access the order form for the Resuscitation Team, who will then post the items requested.

7.11 Resuscitation Drugs

7.11.1 In areas where the resuscitation trolley/grab bag has standardised emergency drugs, replacement drugs must be obtained immediately after use, or if the seal / box appears to have been tampered with, or if it is approaching expiry. In hours this can be done via pharmacy, otherwise contact site practitioners. At NDDH drugs can be ordered via Epic in working hours.

7.11.2 If drugs are missing, report the incident to Ward / Department manager /site practitioner and the Pharmacy department and complete a Datix incident reporting form.

7.12 Defibrillators

7.12.1 Defibrillators are located strategically throughout the acute and community sites.

7.12.2 Defibrillators are standardised throughout the Trust. All emergency defibrillator equipment should be purchased with the consent and agreement of both the Resuscitation Group and Medical Engineering Manager.

7.12.3 Defibrillators should be checked in line with manufacturer's instructions by the relevant ward / departmental staff; any faults being reported to the Medical Engineering team.

7.12.4 While it is highly desirable that those who may be called upon to use an AED should be trained in their use, and keep their skills up to date, circumstances can dictate that no trained operator (or a trained operator whose certificate of training has expired) is present at the site of an emergency. Under these circumstances no restrictions should be placed on any person willing to use an AED. **An AED can be used safely without training** by turning it on and following the voice prompts.

7.12.5 A manual defibrillator can be used in AED mode by any member of staff by turning on the device and following the voice prompts. This is in line with Resuscitation Council (UK) recommendations.

7.12.6 Defibrillation using an automated external defibrillator (AED) can be delivered by any member of staff alongside Basic Life Support by turning on the device and following the voice prompts.

7.12.7 Staff may only perform manual defibrillation if they have received the appropriate training and are competent to do so in their area of practice. If the skill is not used in practice regularly more frequent training should be sought.

7.12.8 It is the staff member performing defibrillation who is responsible for the safety of those around at all time.

7.12.9 **Resuscitation equipment not available on all trolleys/grab bags at acute sites:**
A porter attends all arrest calls. They may be asked to obtain equipment not kept as standard on all resuscitation trolleys.

If intra-osseous (IO) access is required there are IO drills and needles located in:-	
Northern Site	**Eastern Site**
Caroline Thorpe Ward	AMU (West)
Emergency Department	Bramble (Paediatric HDU)
Theatres	Coronary Care Unit
Switchboard	Creedy
	Culm HDU
	Emergency Department
	Intensive Care Unit
	Labour Ward
	Theatres

LUCAS mechanical chest compression devices are located:-	
Northern Site	**Eastern Site**
Emergency Department	Emergency Department
Switchboard	Coronary Care Unit

Emergency tracheostomy equipment is located:-	
Northern Site	**Eastern Site**
Beside the bed of all patients with a tracheostomy	Beside the bed of all patients with a tracheostomy
Intensive Care Unit	Intensive Care Unit
ED Resus	ED Resus
Caroline Thorpe Ward	Otter Ward
In the difficult airway trolley in theatre suites	In the difficult airway trolley in theatre suites

CO_2 monitoring is located:-	
Northern Site	**Eastern Site**
ICU	AMU
Switchboard	Cardiac Cath Labs
ED	Culm HDU
	Emergency Department defibrillators
	Site Practitioners' defibrillator
	With Resus Officers

8 CARDIAC ARREST

8.1 Acute Sites

8.1.1 Any internal telephone may be used to dial 2222 in the event of a suspected cardiac arrest.

8.1.2 Staff must speak clearly and inform the operator they require the cardiac arrest team and give their location.

8.1.3 The operator will then immediately activate the appropriate team bleeps.

8.1.4 Documentation of the cardiac arrest should be entered into the patient's Epic record as soon as possible after the event, using the Resuscitation Documentation note.

8.1.5 See Appendix A for Adult Cardiac Arrest team members.

8.2 Acute Eastern Services

8.2.1 The cardiac arrest team is based within the main building of the Wonford site only.

8.2.2 In the event of cardiac arrest in outlying buildings on the Wonford site off site, (9)999 should be dialled to summon an ambulance.

8.3 Acute Northern Services

8.3.1 The cardiac arrest team is based within the main building of the NDDH site but will also attend calls within the hospital grounds and outlying buildings.

8.3.2 In addition to the Epic documentation a 2222 Resuscitation Data Collection form should be completed for every call at the time of the event. A copy of this should be entered into the patient record and a copy sent to the Resuscitation Team. The Team Leader should sign this form for any cardiac arrest drugs given.

8.3.3 When working in areas with limited access staff should ensure that emergency teams are able to reach them by unlocking doors.

8.4 Adult cardiac arrest and medical emergency team leader

8.4.1 The medical registrar or equivalent would normally assume the role as team leader.

8.4.2 The cardiac arrest team leader will have a current Resuscitation Council UK accredited Advanced Life Support qualification.

8.4.3 The cardiac arrest team leader will co-ordinate the resuscitation process and will be responsible for decision making, whilst including the team.

8.4.4 In the event of simultaneous cardiac arrests or emergencies the cardiac arrest team leader will designate responders to the emergency and obtain anaesthetic help by contacting the 2nd on call anaesthetist via switchboard.

8.4.5 Although the immediate response will be led by the Medical Emergency / Cardiac Arrest team, the responsibility for ongoing care rests with the patient's own clinical team, who also provide speciality expertise. It is vital, therefore, that the most senior available member of the team is notified as soon as possible and should attend urgently.

8.5 Anaesthetic cover for cardiac arrests

8.5.1 On the Eastern (Wonford) site the anaesthetic department does not guarantee that an anaesthetist will be available for every cardiac arrest call. However; a representative from critical care will always attend (usually ICU doctor / ACCP).

8.5.2 If the attending clinical team feel that urgent senior critical care input is required this should be facilitated by the attending critical care staff. The team should also feel empowered to contact the duty consultant intensivist via switchboard (using 2222 if necessary).

8.5.3 On the Northern acute site adult cardiac arrests are attended by an intensive care doctor who may not possess advanced airway skills, but with access to senior support.

8.6 Paediatric cardiac arrest

8.6.1 In the event of a paediatric respiratory/cardiac arrest, staff should dial 2222 and state that it is a paediatric cardiac arrest/emergency and location where the team is required. Switchboard will put out the paediatric cardiac arrest call.

8.6.2 See Appendix A for Paediatric Cardiac Arrest team members.

8.6.3 At least one member of the paediatric resuscitation team should possess a valid certificate in Advanced Paediatric Life Support (APLS) or European Paediatric Advanced Life Support (EPALS).

8.6.4 Parents or carers of a child in cardiac arrest should be offered the opportunity to be present during the resuscitation event and be supported by a registered member of the team.

8.6.5 Parents or carers of high-risk children will be offered resuscitation training.

8.7 Neonatal emergency

8.7.1 In the event of a neonatal emergency, dial 2222 and clearly state that you require the neonatal emergency team and location where the team is required. The switchboard will then immediately activate the neonatal emergency team bleeps.

8.7.2 See Appendix A for Neonatal Emergency team members.

8.7.3 Ensure a record completion of the 2222 event is made in the patient's Epic record as soon as possible after the resuscitation attempt.

8.7.4 Training in newborn life support, assessment and recognition of the sick neonate will be available for clinical staff that are expected to attempt resuscitation of neonates.

8.7.5 Parents or carers on the Neonatal Unit may be offered infant resuscitation training.

8.8 Obstetric / maternal cardiac arrest

8.8.1 If the individual is pregnant or recently delivered the callers on the main Eastern site should state "Maternal cardiac arrest".

8.8.2 If the individual is pregnant or recently delivered the callers on the Northern site should request **both** "adult cardiac arrest team, and obstetric team".

8.8.3 See Appendix A for Obstetric / Maternal Emergency team members.

8.9 Community sites

8.9.1 If a patient has a cardiac arrest at a community hospital, clinic, health-centre / GP practice, patients' home or any other site where staff may be working, staff must dial (9)999 for an emergency ambulance, and commence CPR.

8.9.2 If staff do not accompany the patient to hospital then handover should be telephoned through to the receiving unit and documented in the patient's Epic record.

8.9.3 There will be some people encountered in the community setting for whom attempting CPR is clearly inappropriate (for example, a person in the advanced stages of a terminal illness where death is imminent and unavoidable and CPR would not be successful) but for whom no formal CPR decision has been made and recorded. There may also be cases where healthcare professionals discover patients with features of irreversible death – for example, rigor mortis. In such circumstances, any healthcare professional that makes a carefully considered decision not to start CPR should be supported by their senior colleagues, employers and professional bodies. Decisions relating to cardiopulmonary resuscitation published 2016 Joint BMA, Resuscitation Council and Royal College of Nursing 3rd edition.

8.9.4 Staff working in patients' own homes must ensure personal safety and follow Basic Life Support algorithm where possible. The staff member should dial 999 for assistance before commencing resuscitation.

8.10 Medical emergencies and cardiac arrest in non-clinical areas

8.10.1 When a medical emergency or cardiac arrest occurs in a non-clinical area, hospital staff should provide instant resuscitation within the scope of their training / competence including basic life support, advanced life support and defibrillation.

8.10.2 Eastern staff should follow their separate policy on this: https://hub.exe.nhs.uk/_resources/assets/attachment/full/0/6505.pdf

8.10.3 Automated External Defibrillators (AEDs) have been installed in various locations around the Eastern Site. The cabinets also contain a bag valve mask (BVM). When the cabinet door is opened, the alarm will sound and the lights will flash. The alarm will sound even if the door is closed and continues until turned off by a site practitioner or member of the general services staff. Site practitioners can be contacted via switchboard.

8.10.4 The location of these devices can be identified by an AED wall sign and easily recognisable white cabinets.

9 SPECIAL CIRCUMSTANCES

9.1 Resuscitation of a patient with laryngectomy or tracheostomy

9.1.1 All staff involved in the care of patients with a laryngectomy or tracheostomy should be aware of the procedure for managing the patient's airway and breathing during resuscitation.

9.1.2 The National Tracheostomy Safety Project provides effective guidelines for care of such patient groups.

9.1.3 Additional airway adjuncts must be kept on the resuscitation trolley in clinical areas caring for this patient group.

9.1.4 Patients admitted with a tracheostomy / laryngectomy should have a box at their bedside with appropriate kit to be used in an emergency.

9.2 Anaphylaxis

9.2.1 All employees who are required to provide injections, immunisation programmes and other pharmacological interventions must be proficient in the recognition and immediate treatment protocols of anaphylaxis management.

9.2.2 Under Regulation 238 of the Human Medicines Regulation 2012 schedule 19 up to 1 mg of Adrenaline 1:1000 can be given intramuscularly in two 500 mcg doses (administered five minutes apart) for life saving intervention when anaphylaxis is evident. **This does not require prescription.**

9.2.3 Registered practitioners are able to administer the adrenaline in accordance with their competency and current Resuscitation Council Guidelines.

9.2.4 All other clinical staff must be proficient in the recognition of anaphylaxis and competent in raising the necessary alarm, i.e. the summoning of the Resuscitation Team within the acute hospital setting, and emergency ambulance in all other settings.

9.2.5 Community staff with appropriate training may administer a pre-filled injectable device (e.g. EpiPen), and all staff may assist a patient to administer their own. Staff can seek emergency assistance via 999, and are protected under Schedule 19 for administration with, or without, this source of advice.

9.3 **Resuscitation in patients who have received a radioactive isotope (Eastern services)**

9.3.1 There are 2 uses of radioactive isotopes in the Trust – diagnostic and therapeutic. All patients have close contact restrictions to some extent after administration, for up to 24 hours for diagnostic patients and up to 23 days for patients treated with radioactive iodine.

9.3.2 Therapeutic patients are treated as either day cases or inpatients in sideroom 4 on Yeo ward. The staff on Yeo are trained in the care of these patients and in the event of resuscitation contingency plan RTP-P-31 will be followed, the staff will assist the resus team in following the required procedures. See Appendix B.

9.3.3 Patients who have had a diagnostic scan, or who have been discharged after a therapy, are less radioactive but precautions should be taken to minimise dose to staff. These patients should be wearing a yellow wristband or carrying a yellow card detailing their contact restrictions. In some circumstances they may also have an FYI flag on Epic indicating that they are radioactive. See Appendix C.

9.3.4 Employees who are pregnant or breastfeeding must not be involved in the resuscitation of patients who have received a radioactive isotope. The staff on Yeo will advise accordingly.

9.3.5 Further information can be found on the Medical Physics intranet page or by contacting ext 2125/2123.

10 TREATMENT ESCALATION PLAN AND RESUSCITATION DECISIONS

10.1 **All patients, staff and visitors who have an unexpected cardiopulmonary arrest should be immediately resuscitated, and the cardiac arrest team called, unless an explicit and corroborated decision has already been made not to do so.**

10.2 All advance decisions relating to CPR must be discussed with the patient or a patient's representative by a senior clinician and clearly documented.

10.3 Sometimes people who lack capacity to consent will require emergency medical treatment to save their life or prevent them from serious harm. In these situations, what steps are 'reasonable' will differ to those in non-urgent cases. In emergencies, it will almost always be in the person's best interests to give urgent treatment without delay. One exception to this is when the healthcare staff giving treatment are satisfied that an Advance Decision to Refuse Treatment exists, for example a Do Not Attempt CPR (DNACPR) Instruction or a Treatment Escalation Plan (TEP) has been agreed.

10.4 If a person is being assessed for serious medical treatment and they lack capacity, and have no suitable representative, an independent mental capacity advocate (IMCA)

must be instructed without delay. The only exception for the need to instruct an IMCA is if there is an emergency situation requiring an urgent decision to be made, for example, to save a person's life or for a resuscitation decision. However, once the emergency has passed, there is a duty to instruct an IMCA for any subsequent serious medical treatment decisions. Decisions regarding resuscitation are considered serious medical treatment and require an IMCA if there is no other advocate and the person lacks capacity. **RCUK guidance.**

10.5 No single factor, such as age or disability, should be used making decisions regarding DNACPR. Clinicians should use all available information when making a DNACPR decision and should follow national guidance (from the British Medical Association, Resuscitation Council UK, and General Medical Council)

10.6 If a clinician considers that CPR will not work, the patient cannot expect or request them to provide CPR. This said, the patient is still entitled to know that this decision has been made and the basis for it. Some individuals may have cultural or religious concerns regarding these decisions and discussions (e.g. concerns about 'choosing to die'). Staff have 24-hour access to chaplaincy services to support patients or relatives with ethical, religious or cultural concerns.

10.7 Once a decision relating to resuscitation and ceiling of treatment has been made a senior clinician should record that decision by completing a TEP via the Devon & Cornwall Care Record (DCCR).

10.8 With the patient's permission a 'DNACPR' decision should be explained to and discussed with the relatives as soon as is practical and this conversation documented on the digital TEP form.

10.9 Any member of the medical or nursing teams looking after the patient who disagrees with any element of the Treatment Escalation Plan is encouraged to immediately communicate their concern to the most senior doctor or nurse available. This person will take whatever immediate action is appropriate and refer the matter to the patient's consultant, who has the ultimate responsibility. If this is not appropriate or successful a second opinion should be sought and documented in the patients notes.

10.10 The decisions recorded on a TEP should be reviewed if the patients clinical condition changes. If appropriate, a new TEP should be created, which will cancel the previous version.

10.11 A DNACPR order on the TEP should be reviewed prior to any anaesthetic or other significant procedure if appropriate - e.g. theatre/Cath Lab. This should be communicated to the patient and next of kin. It should also be referenced in the consent form and a note made on Epic. Suspension of a TEP in this way is only valid within the immediate environment (e.g. theatre) and any more prolonged change in resuscitation should be made by re-writing the TEP on the DCCR.

10.12 All staff involved with the patient should be aware of any changes to the DNACPR order.

10.13 Escalation status must be reviewed again prior to discharge. Any patient who has an active TEP (i.e. not the default) should receive a paper copy of that order (to be printed on discharge). The 'live' version of the TEP will always be on the DCCR and this takes priority at all times.

10.14 The patient (unless the senior clinician feels that this would harmful to the patient) and family / carers should be aware of the content of the TEP.

10.15 All staff including ambulance crews should be aware of the patient's resuscitation status.

10.16 If in doubt, always start resuscitation.

10.17 **Mental Capacity Act Practice Guidance** provides a legal framework for acting and making decisions on behalf of people aged 18 (in certain circumstances age 16) and those who lack the mental capacity to act or make decisions themselves. It provides clear guidelines for carers and professionals about who can take decisions, and in which situations.

10.18 Responsibility for TEP decision making rests with the senior responsible clinician, usually the patient's general practitioner or consultant. Decision making can be delegated to any competent member of registered staff, but should be the most senior clinical available. In the acute hospital setting this will usually be a consultant or registrar.

10.19 Teams should ensure that members needing to complete TEP documentation can demonstrate sound understanding of the principles of TEP and factors influencing decision making, including but not limited to capacity assessment, the process of resuscitation, and implications for recovery. They should also demonstrate the ability to undertake conversations compassionately and sensitively.

10.20 Teams should formalise arrangements for the recording of individual competency in accordance with the Integrated Care Board guidance, and ongoing review of this competency (through continuous professional development, appraisal, or other appropriate mechanism). This policy should be reviewed via the Resuscitation Group. Responsibility for alignment to the policy, and training and supervision of individual staff, rests with their clinical team and line management.

10.21 TEP forms must not be completed by non-registered staff, or by those with clinical dependence (for example; medical students, physician associates).

11 RELATIVES WITNESSING RESUSCITATION ATTEMPTS

11.1 Relatives or next of kin should be offered the opportunity to stay during resuscitation attempts. Although this may be uncomfortable for medical and nursing staff, research suggests that it may help the grieving process if the patient subsequently dies. If relatives are present, a registered member of staff should be available throughout the process to support and explain the resuscitation to them. The relatives should be free to leave at any time (see also 8.5.4)

12 POST RESUSCITATION CARE AND PATIENT TRANSFERS

12.1 The cardiac arrest team leader remains responsible for the patient until the patient is transferred to a higher level of care or handed back to the original team. They are also responsible for ensuring that the events and treatment are documented accurately in the patients' notes.

12.2 Safe transfer of the patient post cardiac arrest must be undertaken. From Community locations this will involve the ambulance service via (9)999. Within the acute hospitals, this should involve staff who have had training in the transfer of sick patients, usually a member of the critical care department.

12.3 Portable equipment for safe transfer of the patient should be collected from the appropriate area for your site and checked prior to transfer of the patient, and must include capnography for intubated patients.

12.4 Oxygen for transfer can be obtained via the portering staff.

12.5 Best practice is for the staff involved in a resuscitation attempt to debrief afterwards. If a member of staff needs further support they can contact the resuscitation officers and/or Occupational Health department.

13 SAFETY

13.1 Manual Handling

13.1.1 In situations where the collapsed patient is on the floor, in a chair or in a restricted / confined space the organisational guidelines for the movement of the patient must be followed to minimise the risks of manual handling and related injuries to both staff and the patient. Also refer to the Resuscitation Council (UK) (2009) statement and the Trusts Moving and Handling procedures (including bariatrics).

13.2 Infection Control

13.2.1 Isolated cases of infection to rescuer have been reported; therefore, mouth-to-mouth ventilation should not be performed by staff within clinical settings.

13.2.2 If no bag-valve-mask is available, continuous chest compressions should be delivered until further assistance/equipment arrives.

13.2.3 Personal Protective Equipment should be worn in line with current Infection Control Guidelines, personal risk assessment and the area in which the event takes place.

13.2.4 Universal Precautions should be employed when dealing with body fluids (please see the Trust Infection Control Policies for Eastern and Northern). Management of a resuscitation emergency is no exception.

13.3 Radioactive isotopes

13.3.1 Appendix C contains guidance for the management of resuscitation for patients who have recently received radioactive isotope. See 9.3 for more details

14 RESUSCITATION EXPERTISE IN THE WIDER COMMUNITY

14.1 The Resuscitation Council UK recommend that acute providers offer formal advice and support to other local healthcare organisations who do not have the necessary specialist expertise in resuscitation. The Resuscitation Group will oversee any such requests and ensure that appropriate expertise is offered.

14.2 Royal Devon takes seriously its wider social responsibility, and partnership working to improve the health of our communities is a core Trust objective. The Resuscitation Group will ensure that the expertise of specialists in resuscitation contributes to support for communities across the region, in conjunction with the Peninsula Cardiac Network.

14.3 The Resuscitation Group will ensure that the Trust has mechanisms to identify and support survivors of sudden cardiac arrest and those affected by it (sometimes called co-survivors)

15 ARCHIVING ARRANGEMENTS

The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely.

16 PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY / STRATEGY

16.1 The Resuscitation Officers will review all cardiac arrest calls (2222) and collect data for submission to the National Cardiac Arrest Audit (NCAA). They may also collect baseline process data for all MET calls (Eastern Services) and /or DART calls (Northern Services). Clinical staff in Northern Services are required to complete a Resuscitation Data Collection Form for all 2222 calls. They will not carry out a detailed review of these cases – where there is clinical concern this should be escalated to the Clinical Lead for the Resuscitation Group and the Matron for Patient Safety via the Datix System, or informally to the Consultant responsible for the patient's care or the Senior Nurse for the area. Concerns can also be raised via the Freedom to Speak Up Guardians.

16.2 To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Cardiac arrest audit data will be collected after each resuscitation	Database maintained by the Resuscitation Officers	Via National Cardiac Arrest Audit. The report of which will be discussed at the Resuscitation group
A qualitative review of 2222 calls will be performed on an on-going basis to identify quality and training issues.	Database maintained by the Resuscitation Officers	All results will be presented and discussed at the Resuscitation Group

Any specific learning outcomes and issues will be shared with the Governance Manager(s) for the appropriate Care Groups / division(s).	Datix incident forms completed by RO. Clinical Lead Resuscitation Group notified	These incidents will be investigated by Care Groups and discussed at Care group/divisional governance
Resuscitation equipment including trolleys and grab bags will be audited by the Resuscitation Department annually and results reviewed by the Resuscitation Group, in order to identify areas of non-compliance and assist wards / departments to improve practice.	Minutes of the Resuscitation Group meetings.	Any issues noted will be reported to the Patient Safety and Mortality Group
Care Groups will monitor attendance rates for BLS training (adult, paediatric, neonatal) and put in place action plans to rectify any problems and notify the resuscitation officers.	Learn+ Records.	This will be monitored by the Resuscitation Team, and area managers.
Information on the analysis of audit data, benchmarking and the identification of areas of risk will form part of the resuscitation group reports to the Patient Safety Group		Minutes of the Resuscitation Group and Clinical Effectiveness Committee meetings.

17 REFERENCES

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18 FURTHER INFORMATION

19 APPENDIX A: Emergency team personnel

Arrest teams	Eastern services	Northern services
Adult team	• Medical ST3 or above (or equivalent) • SHO-tier & F1 Doctor (on call for acute medicine) • Anaesthetic/intensive care doctor or Advanced Critical Care Practitioner (ACCP) • Site Practitioner (during normal; working hours if available, all calls out of normal working hours, and non-clinical areas) • Porter • Resuscitation Officers (during normal working hours and if available)	• Medical ST3 or above (or equivalent) • SHO-tier & F1 Doctor (on call for acute medicine) • Anaesthetic/intensive care SHO-tier doctor • Clinical Site Manager (during normal working hours if available, all calls out of normal working hours, and non-clinical areas) • Porter • Cardiographer (during normal working hours) • Resuscitation Officers (during normal working hours and if available)
Paediatric team	• Paediatric specialist registrar • Paediatric SHO-tier doctor • Bramble SHO-tier doctor • Most senior critical care doctor available (may be ICU doctor or anaesthetist) • Paediatric Nurse Coordinator • Neonatal registrar • Site Practitioner • Resuscitation officers (during normal working hours and if available) • Porter	• Paediatric consultant on call • Paediatric specialist registrar • Clinical Site Manager (during normal working hours if available, out of normal working hours and non-clinical areas only) • Anaesthetic/ICU SHO-tier doctor ICU • Senior anaesthetist • Paediatric SHO Neonatal • Resuscitation officers (during normal working hours and if available) • Porter
Neonatal team	• Neonatal registrar • Neonatal SHO-tier doctor • Neonatal nurse • Delivery suite co-ordinator	• Neonatal SHO-tier doctor • SCBU nurse • Resuscitation Officer if available
Maternal team	The adult cardiac arrest team as detailed above and also • on-call obstetric registrar and SHO-tier doctor • Labour ward co-ordinator • Obstetric consultant • Neonatal registrar	• Adult cardiac arrest team (as detailed above) • Neonatal team (as detailed above) • Obstetric emergency team (as detailed below)

	• Senior bleep holder neonatal nurse • Labour ward anaesthetist • Senior anaesthetist on call	
Obstetric emergency team	n/a	• Obs & gynae consultant (in hours) • Senior anaesthetist on call • Anaesthetic SHO ICU • Obs & gynae staff grade • Obs & gynae SHO • ODP on call • Scrub nurse/theatre co-ordinator/C pod • Neonatal paediatric SHO • Senior midwife • Porter

20 APPENDIX B: Emergency treatment of patients who have received radioactive isotope

From the local rules:

1.1 To limit the risk of contamination, as many staff as possible must wear protective clothing. Overshoes, gloves and aprons are kept outside the room.

1.2 Disposable equipment must be used where possible. The resuscitation trolley is kept in the corridor outside side room 4.

1.3 Disposable contaminated items must be disposed of in the bin within the room.

1.4 Reusable items which could potentially be contaminated must be bagged, labelled and stored in B168 until monitored by a physicist

1.5 On leaving the room protective clothing must be removed and disposed of in the room's bin.

1.6 Staff must be monitored for contamination by a physicist, trained nurse or trained housekeeper following **RTP-WI-159**. All readings must be recorded on the record sheet and the decontamination instructions must be followed as necessary

1.7 Following the incident a list of the staff involved and estimates of the times spent at different distances must be recorded on the "Record of Radioactive Incident – Yeo ward" sheet (**RTP-F-323**) in the Local Rules folder. Submit a copy to the RPS.

1.8 Medical Physics must be informed of the incident as soon as possible.

1.9 If there were any spills of bodily fluids, staff involved in the incident must have their thyroid monitored by Medical Physics between 8 hours and 3 days after the incident following **RTP-WI-181**.

1.10 The event must be recorded on Datix, including a list of the staff involved and estimates of the time each individual spent at different distances from the patient.

21 APPENDIX C: Routine care of patients who have received radioactive isotope

- Wear gloves and disposable aprons (+ overshoes if practicable), to reduce the risk of radioactive contamination from bodily fluids
- Keep more than 1 metre away from the patient where possible
- Keep time close to the patient as low as possible
- Consider rotating staff if prolonged close contact is required
- Contact Medical Physics (x2125/2123) to ask for assistance with monitoring for radioactive contamination. Or, if out of hours, contact Yeo ward who have a radiation monitor outside side room 4 and are trained in monitoring themselves for contamination
- Keep all laundry and waste secured in a low occupancy area until Medical Physics can attend
- Keep all non-disposable equipment separate until monitored and decontaminated by Medical Physics

Please note that it is essential that Medical Physics are informed of the incident so that appropriate monitoring and dose estimates are carried out. Also, in the event of death of a patient with radioactive restrictions a Trust RPA must be contacted via Medical Physics (x2125/2123) for advice.

22 APPENDIX D: NEWS2 Clinical response for acute hospitals

Chart 4: Clinical response to the NEWS trigger thresholds

NEW score	Frequency of monitoring	Clinical response
0	Minimum 12 hourly	• Continue routine NEWS monitoring
Total 1–4	Minimum 4–6 hourly	• Inform registered nurse, who must assess the patient • Registered nurse decides whether increased frequency of monitoring and/or escalation of care is required
3 in single parameter	Minimum 1 hourly	• Registered nurse to inform medical team caring for the patient, who will review and decide whether escalation of care is necessary
Total 5 or more Urgent response threshold	Minimum 1 hourly	• Registered nurse to immediately inform the medical team caring for the patient • Registered nurse to request urgent assessment by a clinician or team with core competencies in the care of acutely ill patients • Provide clinical care in an environment with monitoring facilities
Total 7 or more Emergency response threshold	Continuous monitoring of vital signs	• Registered nurse to immediately inform the medical team caring for the patient – this should be at least at specialist registrar level • Emergency assessment by a team with critical care competencies, including practitioner(s) with advanced airway management skills • Consider transfer of care to a level 2 or 3 clinical care facility, ie higher-dependency unit or ICU • Clinical care in an environment with monitoring facilities

23 APPENDIX E: Community NEWS2 response and observation for the deteriorating patient

The aim of this guidance is to set the minimum standard and frequency for monitoring and recording adult patients' vital signs using the NEWS2 tool in their own home, community hospital wards and outpatient areas such as podiatry, ambulatory clinics and MSK services.

Community Services have found the NEWS2 tool and SBAR a beneficial communication tool during formal escalation of the deteriorating patient. It supports community clinicians to recognise signs of deterioration alongside their skilled knowledge of clinical assessment, supporting the clinician to speak a 'universal language' to convey escalation of a deteriorating patient using a structured method of communication.

NEWS 2 scoring:

Physiological Parameter	3	2	1	Score 0	1	2	3
Respiration rate (per minute)	<8 Call 999		9-11	12-20		21-24	>25
SpO2 Scale 1 (%)	<91	92-93	94-95	>96			
SpO2 Scale 2 (%)	<83	84-85	86-87	88-92 >93 on air	93-94 on Oxygen	95-96 on oxygen	>97 on oxygen
Air or oxygen?		Oxygen		Air			
Systolic blood Pressure(mmHg)	<90	91-100	101-110	111-219			>220
Pulse (per minute)	<40		41-50	51-90	91-110	111-130	>131
Consciousness				Alert			CVP U = Call 999
Temperature	<35		35.1-36	36.1-38	38.1-39	>39.1	

Codes for Oxygen therapy and CVPU NEWS2 observation chart	
A (Breathing Air) N (Nasal Cannula) SM (Simple Mask) NIV (patient on None Invasive Ventilation system) CP (CPAP mask) OTH (Other, please specify.....)	CVPU C = New confusion V = responsive to voice P = responsive to pain only U = unresponsive, a medical emergency

When taking the patient's pulse do this manually, as a machine will not detect volume or rhythm. Perform manual BP if unable to gain a reading on the machine. Respiratory rate is widely accepted as being the most sensitive basic observation in detecting deterioration in the patient's condition.

Community Hospitals

Frequency of NEWS2 Observations
All inpatients on a community hospital ward will have there NEWS2 recorded at a minimum of 12 hourly.

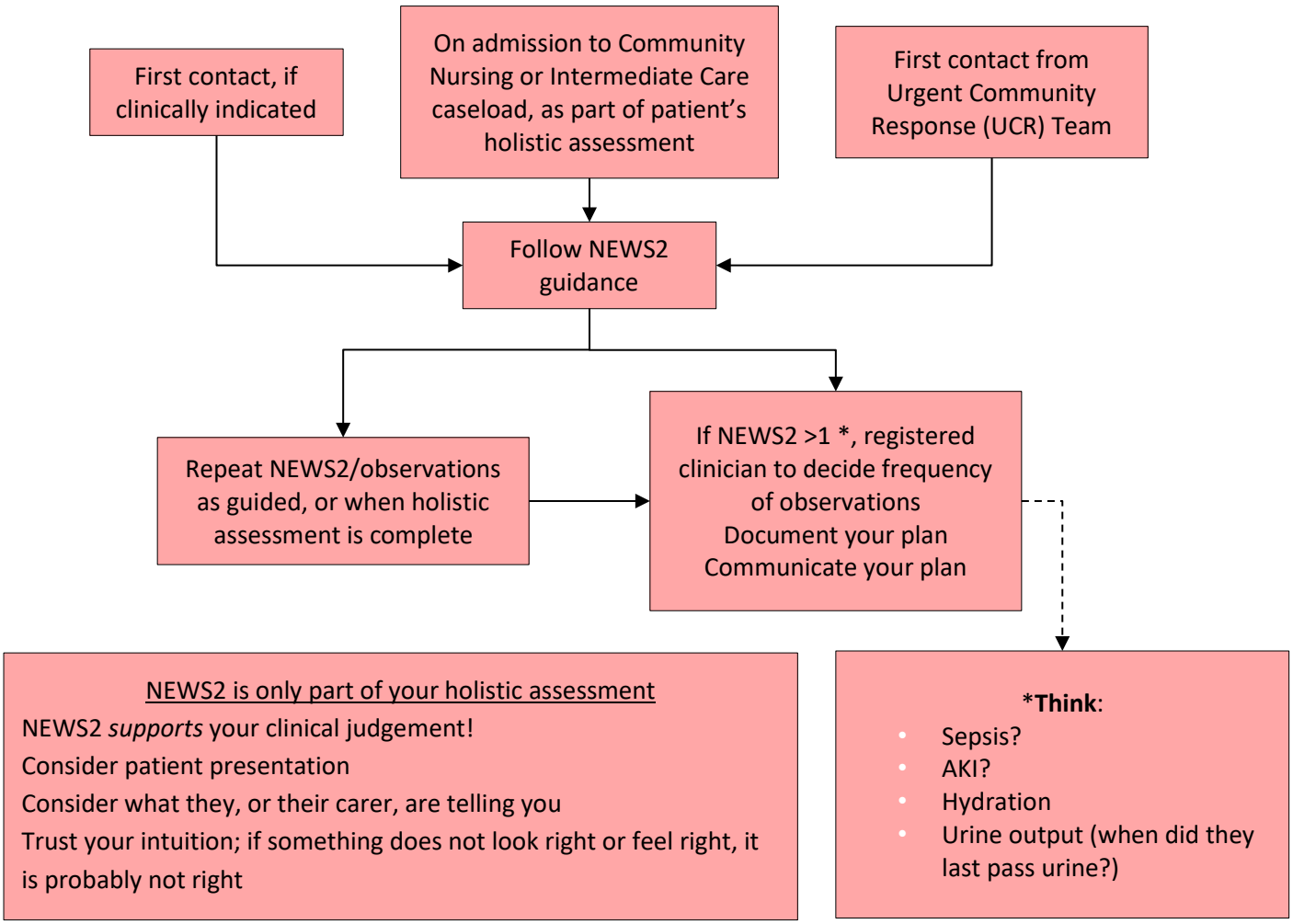
Patients who may not require physiological observations

Discontinue observations for those patients at the end of their life unless the scoring will alter the course of their treatment, as agreed by the MDT.

If there is any doubt as to whether observations should be carried out the clinician should carry out observations document and escalate as necessary.

NEW Score	Risk	Frequency of Monitoring	Clinical Response & Actions
0	**Low**	Minimum 12 hourly	• Continue routine NEWS monitoring
1-4	**Low**	Minimum 4-6 hourly	• Inform registered nurse, who must assess the patient • Registered nurse decides whether increased frequency of monitoring and/or escalation of care is required
3 in a single parameter	**Low-Medium**	Minimum 1 hourly	• Registered nurse to inform medical team caring for the patient, who will review and decide whether escalation of care is necessary
5-6	**Medium**	Minimum 1 hourly	• Registered nurse to immediately inform the local clinical team caring for the patient or out of hours/med SpR as per local policy
7+	**High**	Continuous monitoring of vital signs	• Registered nurse to immediately call 999 and immediately inform the local clinical team caring for the patient

Other community contacts, including domiciliary and outpatient areas



Clinicians who are unable to perform NEWS2 please continue to escalate rapidly deteriorating patients through 999, and any other clinical concerns or signs of deterioration to the patients GP or if appropriate escalate to senior clinician in the team and patient safety handover.

Patients who may not require physiological observersations

Patients receiving task-based care (e.g.: phlebotomy, wound care) from an RDUH service may not routinely have physiological health observations carried out.

Patients identified as being at the end of their life will not need observations carried out. Consider the patient's TEP form.

If there is any doubt as to whether observations should be carried out the clinician should carry out observations document and escalate as necessary to the patients General Practitioner (GP).

NEWS2 guidance for specific community teams:

Community Nurses, Intermediate Care, Community Therapy Team (when trained to perform NEWS2)

NEW Score	Risk	Frequency of Monitoring	Clinical Response & Actions
0	**Low**	Perform at first visit then. For future visits where vital signs are required always calculate a NEWS2 score	Likely stable to remain at home. • Escalate if any clinical concerns
1-4	**Low**	Ensure the relevant clinical response and agreed actions have been undertaken within 12-24 hours and document in patient's electronic patient records	If NEWS2 is different from baseline: • Inform registered nurse to assess patient or • Discuss with Community Matron / ACP / GP/111 (for Urgent Care Nursing Team) • Escalate at patient safety handover*
3 in a single parameter	**Low-Medium**	Repeat prior to leaving patient's home Escalate if new confusion or if patient is not rousable.	If NEWS2 is different from baseline: • Request review by Community Matron / ACP / GP/111 (for Urgent Care Nursing Team) • Escalate at patient safety handover*
5-6	**Medium**	Every 15 minutes until review or advice obtained	• Request urgent review by Community Matron / ACP / GP/111 (for Urgent Care Nursing Team) • Urgent transfer to hospital may be required • Escalate at patient safety handover*
7+	**High**	Repeat observations every 15 mins until emergency services arrive	•Call 999 for emergency transfer to hospital unless contrary to advance care plan and check TEP form or an Advanced Decision to Refuse Treatment stating that the individual is not for resuscitation. •Escalate at patient safety handover*

*If patient is not for hospital admission, discuss in patient safety handover, and decision to be made on most appropriate team to manage deteriorating patient, consider if appropriate to escalate to UCR

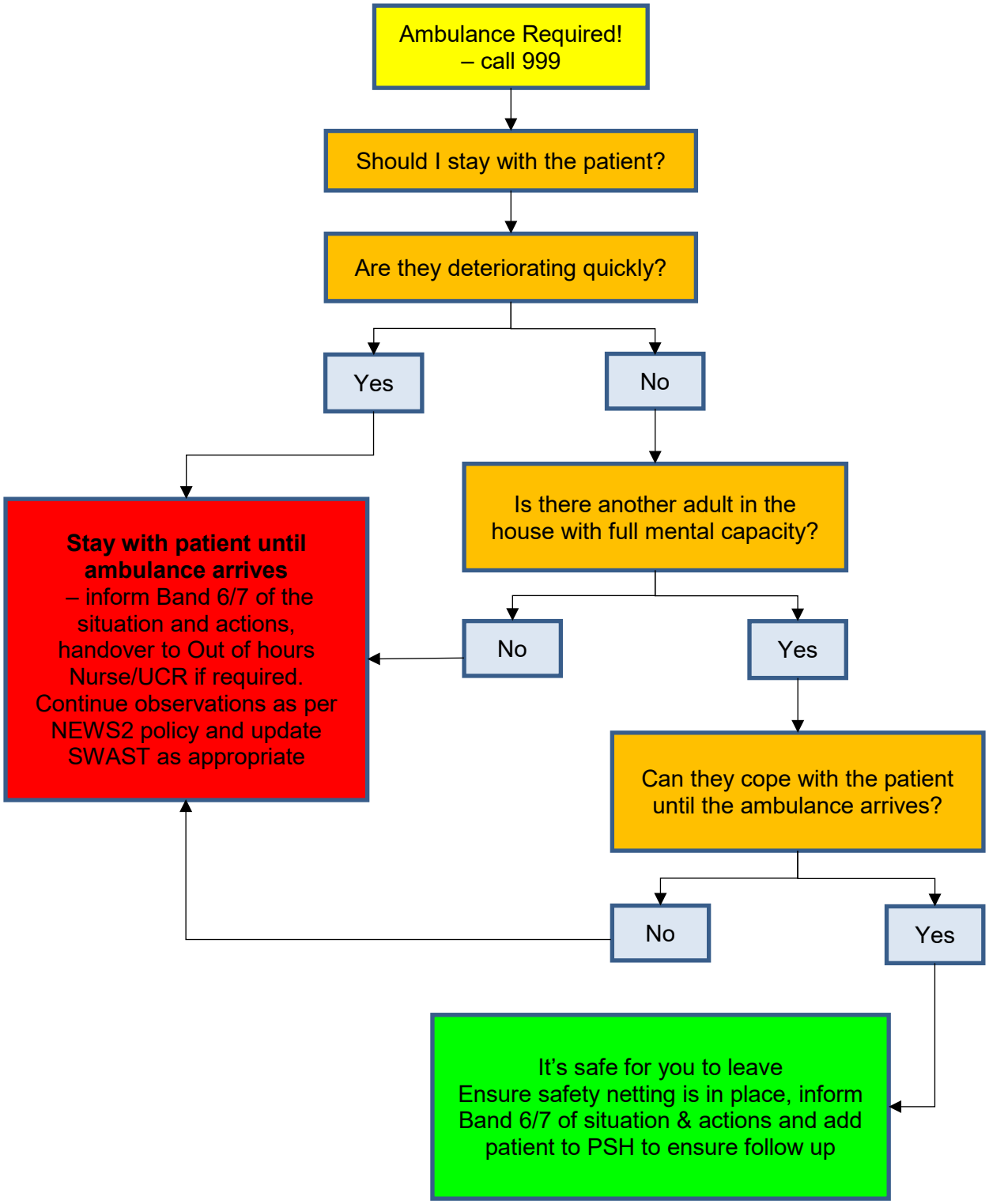
Patients on a Virtual Ward pathway

NEW Score	Risk	Frequency of Monitoring	Clinical Response & Actions
0	**Low**	Perform at first visit then. For future visits where vital signs are required always calculate a NEWS2 score	Likely stable to remain at home. • Escalate if any clinical concerns
1-4	**Low**	Follow advise provided by the Care Coordination HUB clinician	If NEWS2 is different from baseline: • Escalate to Care Co-ordination HUB (add number here) to speak to a clinician within the Virtual Ward Team • Escalate at patient safety handover*
3 in a single parameter	**Low-Medium**	Repeat prior to leaving patient's home Escalate if new confusion or if patient is not rousable.	If NEWS2 is different from baseline: Escalate to Care Co-ordination HUB to speak to a clinician within the Virtual Ward Team
5-6	**Medium**	Every 15 minutes until review or advice obtained	• Request urgent review to Care Co-ordination HUB to speak to a clinician within the Virtual Ward Team • Urgent transfer to hospital may be required • Escalate at patient safety handover*
7+	**High**	Repeat observations every 15 mins until emergency services arrive or advise obtained by care coordination HUB	• Request urgent review to Care Co-ordination HUB to speak to a clinician within the Virtual Ward Team *If unable to get a prompt response from VW team, call 999 for emergency transfer to hospital unless contrary to advance care plan and check TEP form or an Advanced Decision to Refuse Treatment stating that the individual is not for resuscitation.* • Escalate at patient safety handover*

Patients under Urgent Community Response (UCR)

NEW Score	Risk	Frequency of Monitoring	Clinical Response & Actions
0	**Low**	Perform at first visit then. For future visits where vital signs are required always calculate a NEWS2 score	Likely stable to remain at home. • Escalate if any clinical concerns
1-4	**Low**	Ensure the relevant clinical response and agreed actions have been undertaken within 12-24 hours and document in patient's electronic patient records	If NEWS2 is different from baseline: • Inform registered nurse to assess patient or • Discuss with ACP / GP/ Care Co-ordination HUB /111 (for Urgent Care Nursing Team) • Escalate at patient safety handover*
3 in a single parameter	**Low-Medium**	Repeat prior to leaving patient's home Escalate if new confusion or if patient is not rousable.	If NEWS2 is different from baseline: • Request review by Community Matron / ACP / GP/111 (for Urgent Care Nursing Team) • Escalate at patient safety handover*
5-6	**Medium**	Every 15 minutes until review or advice obtained	• Request urgent review by ACP/ GP/111 (for Urgent Care Nursing Team) • Urgent transfer to hospital may be required • Escalate at patient safety handover*
7+	**High**	Repeat observations every 15 mins until emergency services arrive	•Call 999 for emergency transfer to hospital unless contrary to advance care plan and check TEP form or an Advanced Decision to Refuse Treatment stating that the individual is not for resuscitation. •Escalate at patient safety handover*

Ambulance Wait Actions Flow Chart



Communication

To improve communication between clinical staff it is recommended the person calling for advice uses a structured communication tool. SBAR (Situation-Background-Assessment-Recommendation) is the recommended RDUH Trust tool. This is easy to remember in an emergency and ensures essential information is communicated enabling an appropriate timely. Ensure you have all of the physical observations ready to handover to referring clinician.

S

Situation:
I am (name), (X) nurse on ward (X)
I am calling about (patient X)
I am calling because I am concerned that...
(e.g. BP is low/high, pulse is XX, temperature is XX, Early Warning Score is XX)

B

Background:
Patient (X) was admitted on (XX date) with...
(e.g. MI/chest infection)
They have had (X operation/procedure/investigation)
Patient (X)'s condition has changed in the last (XX mins)
Their last set of obs were (XX)
Patient (X)'s normal condition is...
(e.g. alert/drowsy/confused, pain free)

A

Assessment:
I think the problem is (XXX)
And I have...
(e.g. given O_2/analgesia, stopped the infusion)
OR
I am not sure what the problem is but patient (X) is deteriorating
OR
I don't know what's wrong but I am really worried

R

Recommendation:
I need you to...
Come to see the patient in the next (XX mins)
AND
Is there anything I need to do in the mean time?
(e.g. stop the fluid/repeat the obs)

Ask receiver to repeat key information to ensure understanding

The SBAR tool originated from the US Navy and was adapted for use in healthcare by Dr M Leonard and colleagues from Kaiser Permanente, Colorado, USA

24 APPENDIX F: COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the policy	All staff
The key changes if a revised policy/strategy	New policy following merger of North Devon Healthcare Trust and Royal Devon & Exeter Trust
The key objectives	The purpose of this policy is to provide guidance for the planning and implementation of a high-quality resuscitation service to the Trust (acute and community), whilst supporting all Trust activity with regards to issues around resuscitation, recognition of and response to deteriorating patients, and training requirements for staff.
How new staff will be made aware of the policy and manager action	Via the corporate induction process. Communication via the trust intranet. Via Resuscitation training sessions
Specific Issues to be raised with staff	Staff will be made aware on induction and through mandatory training of the new resuscitation policy highlighting key points as above in Key points section.
Training available to staff	Resuscitation Department, mandatory updates, Resuscitation Council (UK) courses, local bespoke education.
Any other requirements	Nil
Issues following Equality Impact Assessment (if any)	Nil
Location of hard / electronic copy of the document etc.	The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet, archived electronic copies will be stored on the Trust's “archived policies” shared drive and will be held indefinitely.

25 APPENDIX G: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	Resuscitation Policy
Division/Directorate and service area	Safety and Risk
Name, job title and contact details of person completing the assessment	Rachel Toft and Dave Robinson, Senior Resuscitation Officers
Date completed:	Sept 2022

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

This policy provides staff with guidance on the responsibilities of the organisation, resuscitation service and staff in the recognition, escalation and management of deteriorating patients and in the management of a cardiopulmonary arrest. This guidance is as recommended by the Resuscitation Council (UK). This policy applies to all staff within the organisation in the acute and community setting.

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☒ Staff ☒ Patients ☒ Other (please specify) Visitors

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☒	☐
Disability	☒	☐
Sex - *including: Transgender, and Pregnancy / Maternity*	☐	☒
Race	☐	☒
Religion / belief	☒	☐

Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒

4. Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to… (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

Homeless may not have up to date medical notes.

5. Do you think the document meets our human rights obligations? ☒
Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:
• ***Fairness –*** *how have you made sure it treat everyone justly?* • ***Respect –*** *how have you made sure it respects everyone as a person?* • ***Equality –*** *how does it give everyone an equal chance to get whatever it is offering?* • ***Dignity –*** *have you made sure it treats everyone with dignity?* • ***Autonomy –*** *Does it enable people to make decisions for themselves?*

6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?

It was noted during the review of the policy that age, disability, religious belief or cultural beliefs may be relevant to the policy. We have amended the policy to make such issues explicit, to ensure these are proactively addressed

7. If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact please note this below and how this will be monitored/addressed.

"Protected characteristic":	
Issue:	
How is this going to be monitored / addressed in the future:	

Group that will be responsible for ensuring this carried out:	